

Standard Operating Procedure

Referral and Transfer of Patients to Another Facility

«Hospital Name» — Supports AAC.1.b

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» identifies, stabilises, documents, and safely transfers a patient who needs a diagnostic or treatment service that falls outside the hospital's defined scope, to a facility that can provide it.

2. Scope

Applies to every patient transfer to an external facility for any service limitation listed in the AAC.1 Required Records (Record 7 — Service Limitation & Referral Pathway), whether the transfer is planned or emergency.

3. Responsibility

Treating Doctor: decides the need for transfer, ensures pre-transfer stabilisation, and completes the referral letter.

Nurse-in-Charge: prepares the patient, confirms documents are complete, and accompanies the patient where clinically indicated.

Transport / Ambulance Coordinator: arranges appropriate transport based on the patient's clinical condition.

Medical Superintendent: is informed of every high-risk or contested transfer and resolves any receiving-facility refusal.

4. Procedure

4.1 Decision to transfer

1. The treating doctor confirms the required service is listed as a defined limitation in Record 7, and identifies the correct referral facility for that limitation.
2. Where the situation is not already covered by a defined pathway, the treating doctor selects the nearest appropriate facility and informs the Medical Superintendent before proceeding, except where delay would endanger the patient.
3. Before finalising the transfer, confirm directly with the receiving facility that it can actually treat the patient's specific condition and is willing to accept the patient — do not rely on assumption or a general reputation for the service.

4.2 Pre-transfer stabilisation

1. Stabilise the patient to the extent possible within the hospital's capability before transfer, per the patient's clinical condition and standard resuscitation/stabilisation practice.
2. For a time-critical transfer (e.g., suspected stroke, STEMI, major trauma), do not delay stabilisation or the referral letter for non-essential paperwork. There is no fixed universal time limit for when remaining paperwork must follow — treat it as urgent and complete it as soon as clinically safe to do so, proportional to how time-critical the transfer

itself was. [Hospital to define: if this hospital wants a specific internal target for completing remaining paperwork, state it here — no external authority mandates a specific number, so any figure used here is this hospital's own operational choice, not a cited requirement.]

4.3 Documentation

1. Complete a referral letter stating: diagnosis, treatment given, current clinical status, vital signs at the time of transfer, and the specific reason for referral.
2. Attach copies of relevant investigations, imaging, and medication administration records.
3. Obtain informed consent for transfer from the patient or attendant. If the patient or attendant refuses transfer, or requests transfer before stabilisation is complete, document the refusal, the risks and benefits explained, and the counselling given.

4.4 Arranging transport

1. Select transport appropriate to clinical severity (basic ambulance, advanced life support ambulance, or other), per the hospital's transport-arrangement protocol.
2. For any patient requiring monitoring or intervention en route, a qualified staff member accompanies the patient.

4.5 Handover at the receiving facility

1. Hand over the patient with a verbal summary plus the written referral letter and all attached documents to the receiving clinician directly — not to administrative staff alone.
2. Obtain and retain acknowledgement of receipt (signature, stamp, or equivalent) from the receiving facility.

4.6 After the transfer

1. Log the transfer in the Patient Transfer Log (Record 14 of the AAC.1 Required Records pack) the same day.
2. Where the receiving facility declines to accept the patient, escalate immediately to the Medical Superintendent and document the refusal and the resolution.

5. Records

Every transfer must be logged in the Patient Transfer Log. This SOP does not create a separate log — it defines how that record gets filled in correctly, and works together with Record 7 (which defines the referral pathway) rather than replacing it.

6. Related Documents

- HCO_AAC_1_v2_REWRITE_DRAFT — AAC.1 policy (this SOP supports AAC.1.b)
- AAC.1 Required Records — Record 7: Service Limitation & Referral Pathway
- AAC.1 Required Records — Record 14: Patient Transfer Log

7. Review

This SOP is reviewed annually, or immediately if a transfer incident or near-miss reveals a gap in the procedure.

8. References

- American College of Emergency Physicians. (2022). Appropriate interfacility patient transfer (Policy statement). <https://www.acep.org/patient-care/policy-statements/appropriate-interfacility-patient-transfer/>